



Anticipatory 'Just in case' Prescribing Guideline for Adults

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Anticipatory 'Just in case' Prescribing Guideline for Adults

This guideline is intended for use by primary care clinicians.

Please seek specialist advice if you are unsure regarding the management/care of the patient at any point

There are some small variations with doses between this document and full local palliative care guidelines available in some ICB areas, but these doses have been clinically agreed by local palliative care specialists. Please note full local guidelines can be found:

- Berkshire West: [Berkshire Adult Palliative Care Guidelines Best Practice \(Section 1+2\)](#)
[Berkshire Adult Palliative Care Guidelines Best Practice \(Section 3\)](#)
- Buckinghamshire: [Palliative Care Symptom Control \(bucksformulary.nhs.uk\)](#)
[Care of the Dying Patient \(bucksformulary.nhs.uk\)](#)

Anticipatory, or 'Just in case', prescribing forms a key part of pro-active end symptom management at the end of life (EoL). It ensures that, in last days/hours of life, there is reduced delay in responding to symptoms. NICE 'Care of dying adults in last days of life' ([NG31](#)) offers guidance on this also.

Prescribing needs to be considered when:

- You think the patient is in the last few weeks of life
- Amber or Red on Palliative care Register (i.e. unstable or likely to change)
- Patient at home on syringe pump
- Patient for whom fast track Continuing Healthcare (CHC) funding has been agreed
- Unstable symptom issues such as
 - Poorly controlled symptoms with concern about absorption
 - Likelihood for vomiting
 - Conservatively managed bowel obstruction
 - History of seizures and potential need to switch to subcutaneous route if loses oral route
 - Medical conditions where medication may be needed by alternative routes when patient unable to swallow
 - Variable ability to swallow

All patients dying at home should have a supply of anticipatory subcutaneous medication but not all patients who are dying will require a syringe pump. A syringe pump should not be prescribed in anticipation but should only be prescribed at the time it is required. Consider using a syringe pump to deliver medicines if more than 2 or 3 doses of any 'as required' (PRN) medicines have been given within 24 hours or if oral route is lost and the patient is on medication that it is appropriate to transition into the subcutaneous route. A syringe pump takes at least 4 hours to establish a steady state drug level in plasma. If the patient is in pain, has nausea/vomiting or is agitated give a when required subcutaneous injection of appropriate medication whilst waiting for the syringe pump to take effect

Seek specialist advice when symptoms persist, patient continues to require frequent PRN medications, if moderate-severe renal or hepatic impairment or if exceeding typical maximum doses. Request advice if there is concern about drug effectiveness, &/or side-effects.

Please ensure a Direction To Administer (DTA) is completed at the time of prescribing these can be completed in EMIS.
Please see [Form: Direction to Administer - Generic Referral form \(clinox.info\)](#) for further information in Oxfordshire.

For Specialist advice, telephone contacts:

Oxfordshire patients:

OUH Palliative Care Hub

9am to 5pm daily: 01865 857036

A senior doctor in Palliative Medicine is available via OUH switchboard for urgent out of hours advice 0300 304 7777

Sue Ryder Team Palliative Care Hub (South Oxfordshire) 8am – 8pm 7 days a week 0330 053 6092

OOH urgent advice contact Royal Berkshire Hospital switchboard for On Call Consultant

Berkshire West patients:

Community patients 9-5, Monday- Friday: **West Berks 01635273720/ Reading 01189505276 / Wokingham 01189787843**

Weekend Daytime and BH on call Clinical Nurse Specialist CNS **07899 915619**

OOH urgent advice contact Royal Berkshire Hospital switchboard for On Call Consultant

Buckinghamshire patients:

Specialist Medical Advice is available 24/7 via Florence Nightingale Hospice 01296 332600

The Guaranteed Provision of Urgent Medication in the Community Scheme details a list of participating pharmacies and the list of palliative care drugs that these pharmacies are guaranteed to stock. Please note that carers and healthcare professionals are not restricted to obtaining palliative care drugs only from the pharmacies taking part in this scheme. Prescriptions for palliative care drugs can be fulfilled by any other community pharmacy. For full details please see [The Guaranteed Provision of Urgent Medication in the Community Scheme](#)

PRESCRIBING GUIDANCE FOR DRUGS ON AN 'AS REQUIRED' BASIS

NAME OF DRUG & INDICATION/SYMPTOMS		AS REQUIRED (PRN) DOSE	FREQUENCY & MINIMUM PRN DOSE INTERVAL	MAXIMUM DOSE in 24hrs before seeking specialist advice	ROUTE	PRESCRIBING COMMENTS	Suggested number of ampoules
MORPHINE SULPHATE injection CD 10mg/ml † OR OXYCODONE injection CD 10mg/ml † 1. Pain 2. Breathlessness (1st line)		Morphine 2.5- 5 mg if opioid naive Oxycodone 1.25-2.5mg if opioid naive If already on regular oral morphine/oxycodone, PRN dose may be higher. Divide total daily oral dose by 2 to calculate 24 hr SC equivalent. Then divide by 6 to calculate SC PRN.	1-2 hourly PRN	Usual maximum is 6 doses of prn in 24hrs Prescribe a maximum dose	SC	Seek specialist advice if: -high doses of opiate eg > 120mg morphine oral/24hrs -on transdermal patch (fentanyl or buprenorphine) -if renal and/or hepatic impairment Use with caution in low eGFR - OXYCODONE usually preferred when eGFR<30	5-10 (Clinical variation may require adjustment of the quantity prescribed)
MIDAZOLAM injection † CD 10mg/2mls	1. Anxiety 2. Restlessness & agitation without evidence of hallucinations 3. Breathlessness (2nd line)	2.5-5mg	2 hourly PRN*	30mg/24h & then review and seek specialist advice	SC	*1 hourly PRN under specialist guidance	5-10 (Clinical variation may require adjustment of the quantity prescribed)
	Risk of seizure	10mg	Once			Can be repeated if seizures persist seek specialist advice	
HYOSCINE BUTYLBROMIDE (BUSCOPAN) injection †20mg/ml 1. Respiratory secretions 2. Abdominal colic/secretions		20mg	4-8 hourly PRN*	1. Respiratory secretions: 60mg/24hrs & then review 2. Abdominal colic/secretions: 120mg/24h & then review	SC	*Seek specialist advice if not controlled. Do not confuse with hyoscine hydrobromide	5-10 (Clinical variation may require adjustment of the quantity prescribed)
HALOPERIDOL injection 5mg/ml † 1. Nausea & vomiting 2. Hallucinations 3. Agitation /delirium OR LEVOMEPRMAZINE injection 25mg/ml 1. Nausea & vomiting 2. Agitation/delirium 3. Severe terminal agitation		0.5-1.5mg	4 hourly PRN if 0.5mg dose OR 1.5mg BD PRN	3mg/24h & then review	SC	(1st line antiemetic) Do not prescribe in Parkinson's disease – seek specialist advice Caution: Dose accumulation can occur due to long half life.	5-10 (Clinical variation may require adjustment of the quantity prescribed)
		6.25-25mg dose range varies according to indication:		25mg/24h & then review. Seek specialist advice if dose above 25mg/24hrs if used for nausea and vomiting and above 50mg/24hrs if used for agitation/delirium.	SC	(2nd line antiemetic) More sedating than haloperidol. Lower end of dose range for nausea & vomiting. Do not prescribe in Parkinson's disease – seek specialist advice. Caution: Dose accumulation can occur due to long half life.	5-10 (Clinical variation may require adjustment of the quantity prescribed)
		1. Nausea & vomiting 6.25mg (lower range)	BD PRN				
		2. Agitation/delirium 6.25-12.5mg	4 hourly PRN				
		3. Severe terminal agitation 12.5-25mg (higher range)	4 hourly PRN				

† denotes the use of medications that are licensed but being used outside of their product licence (off label) but use is 'generally accepted' in palliative care

CD = Control Drug, **SC** = Subcutaneous, **PRN** = As Required, **BD** = twice a day

PRESCRIBING GUIDANCE FOR DRUGS VIA A SYRINGE PUMP CONTINUOUS SUBCUTANEOUS INFUSIONS (CSCI)

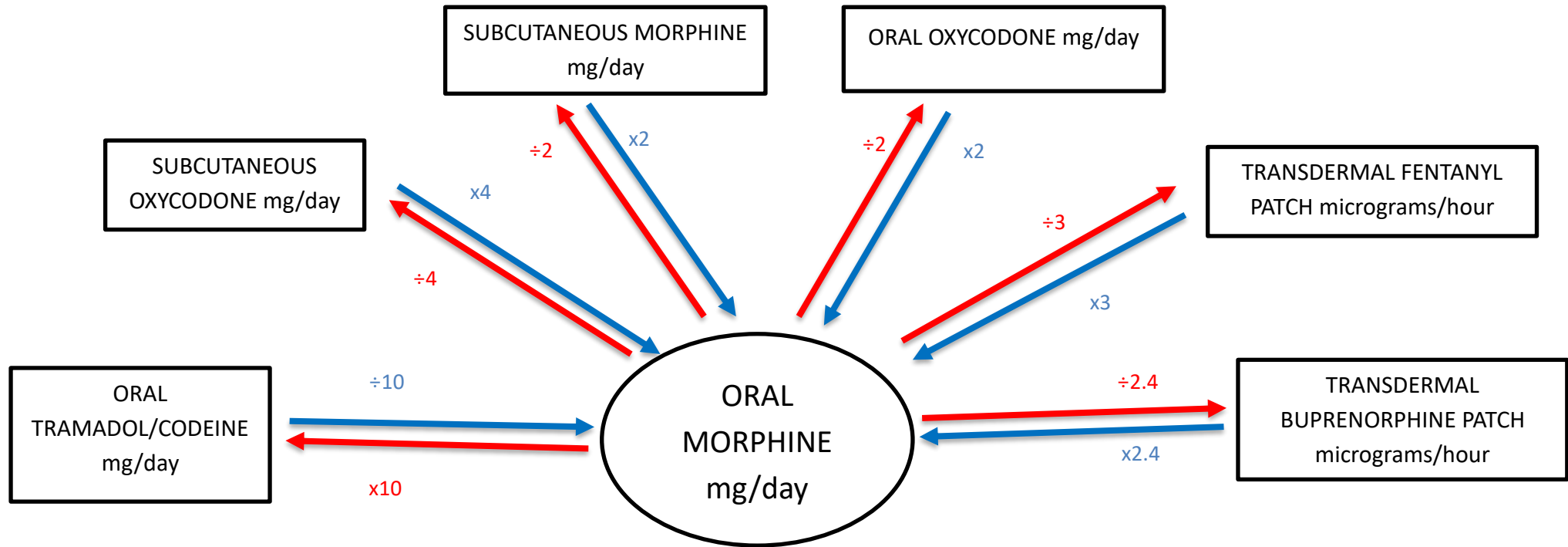
DRUG VIA CSCI	INDICATIONS FOR USE	STARTING DOSE RANGE/24HR	Typical MAXIMUM/24HR	COMMENTS	DILUENT
MORPHINE SULPHATE † CD	Opioid responsive pain Breathlessness 1st line opioid analgesic	Determined by previous opioid requirements	No true ceiling dose (response dependent)	To convert total daily dose of oral morphine to 24h SC syringe pump dose divide by 2. Prescribe SC breakthrough dose of 1/6th of total 24h SC morphine dose in syringe pump.	Water For Injection (WFI)
OXYCODONE † CD	Opioid responsive pain Breathlessness If morphine not tolerated or renal impairment (eGFR less than 30ml/min) 2nd line opioid analgesic	Determined by previous opioid requirements	No true ceiling dose (response dependent)	To convert total daily dose of ORAL oxycodone to 24h SC syringe pump dose divide by 2. Prescribe SC breakthrough dose of 1/6th of total 24h SC oxycodone dose in syringe pump.	WFI
CYCLIZINE †	Nausea & vomiting (N & V) Useful for N & V due to raised intracranial pressure or vestibular disturbance	75mg/24h	150mg/24h	Slows peristalsis in GI tract & acts directly on vomiting centre. Relatively safe antiemetic in Parkinson's disease. Anticholinergic effects of cyclizine block prokinetic action of metoclopramide – do not prescribe concurrently	WFI ONLY
HALOPERIDOL †	Nausea & vomiting Hallucinations/delirium	0.5-1.5mg/24h 1.5-5mg/24h	5mg/24h	Antipsychotic, but at lower doses useful for metabolic/chemical causes of N & V. Do not prescribe in Parkinson's Disease -seek specialist advice.	WFI
METOCLOPRAMIDE †	Nausea & vomiting	30mg/24h	30mg-60mg/24h Please seek specialist advice re higher doses/if symptoms are not controlled as doses above this may be used.	Prokinetic antiemetic useful for N & V caused by gastric stasis. Avoid if complete bowel obstruction or colic. Do not prescribe in Parkinson's Disease -seek specialist advice.	WFI
LEVOMEPRMAZINE	Nausea & vomiting Agitation /sedation Severe agitation	5mg/24h 12.5-25mg/24h 25-50mg/24h	25mg/24h Please seek specialist advice re higher doses/if symptoms are not controlled as doses above this may be used.	Broad spectrum 2nd line antiemetic. More sedating than haloperidol. Sedation & postural hypotension at higher doses. Higher doses sometimes required for severe terminal agitation unresponsive to haloperidol/midazolam. Do not prescribe in Parkinson's disease – seek specialist advice.	WFI or 0.9% Sodium Chloride as reduce risk inflammation
MIDAZOLAM † CD	Anxiety/Agitation/Restlessness Breathlessness	5-15mg/24h	10mg/24h Seek advice if exceed 60mg/24h	Used to replace oral anti-epileptics at EoL. Seek advice in patients losing the oral route who are not thought to be dying. In terminal agitation, always exclude urinary retention & pain.	WFI
	Seizures	20-30mg/24h	Please seek specialist advice re higher doses/if symptoms are not controlled as doses above this may be used.		
HYOSCINE BUTYLBROMIDE † (BUSCOPAN)	Respiratory tract secretions Colic pain Gastrointestinal obstruction	40-60mg/24h	120mg/24h	Give early to prevent build up of respiratory tract secretions. Antispasmodic slows peristalsis & reduces secretions in GI tract.	WFI

† denotes the use of medications that are licensed but being used outside of their product licence (off label) but use is 'generally accepted' in palliative care

CD = Control Drug, SC = Subcutaneous, PRN = As Required

Palliative Care Opioid Conversion Chart

NOTE: When using this chart, calculate the total daily dose of morphine.
Once you have converted from oral morphine, ensure opioid dose is prescribed in divided doses as appropriate.



* The conversions given are comparable doses but there is wide patient intervariability relating to opioid conversion.

Using the chart

- The **RED** arrow **OUTWARD** shows how to convert from morphine to the alternative drug
- The **BLUE** arrow **INWARD** shows how to get the equivalent dose of oral morphine. See overleaf.

In renal impairment (GFR less than 40ml/min)

Morphine and codeine can both accumulate. Consider alternative if eGFR less than 30ml/min.

Alternative opioids of choice are oxycodone oral/scut fentanyl and buprenorphine patches. If using other analgesics, use low doses and increase the dose interval, and regularly monitor and review.

In renal impairment (GFR less than 20ml/min)

Seek specialist advice from the palliative care team.

Palliative Care Opioid Conversion Chart

Opioid Drug Conversions- General Guidelines

- This chart is designed for guidance only. The conversions given are comparable doses but there is wide patient intervariability relating to opioid conversion. For individual patients, response to previous opioids, clinical condition and severity of pain must be taken into consideration.
- The chart is intended to be used as a guide by working through the oral daily dose equivalent of morphine
- Take care if switching from oral to parenteral opioids if clinical concerns regarding oral absorption.
- For patients on higher doses of opioids (morphine > 120mg/day), specialist palliative care assessment and advice should be sought.
- It is good practice to document your rationale for opioid switching.
- When calculating doses always double check your calculation, with someone else if possible.
- Round doses up or down to a sensible number dependent upon the drug and formulations available.
- Remember to prescribe breakthrough doses for PRN use (1/6 of total daily dose).
- To convert to/from methadone – seek specialist advice from the palliative care team

Please be aware that there are several different buprenorphine patches available.

Some are changed **weekly** e.g. Butech and BuTrans, others are changed **twice weekly** e.g. Transtec and Bupeaze (96hrs) or Hapoctasin (72 hrs).

Please see the BNF or Summary of Product Characteristics (available at www.medicines.org.uk) for full details of all available products.

Example calculations from weak opioids to oral morphine

Dose of weak opioid	Calculation to oral morphine	Suggested prescribed total 24 hr daily dose of oral morphine (mg)
Codeine 60mg QDS	Codeine 240 ÷ 10 = 24mg/day	20
Tramadol 100mg QDS	Tramadol 400 ÷ 10 = 40mg / day	40

Example alternative opioid calculations from total daily dose of 80mg oral morphine

Daily 24hr dose of morphine (mg)	Calculation to alternative opioid	Suggested prescribed dose of alternative opioid
80	Fentanyl patch $80 \div 3 = 26.66$ microgram/hour	Fentanyl patch 25microgram/hour
80	Subcutaneous oxycodone $80 \div 4 = 20$ mg/day	Subcutaneous oxycodone 20 mg per 24 hours
80	Oral oxycodone $80 \div 2 = 40$ mg/day	Oral oxycodone S/R 20 mg BD
80	Buprenorphine patch $80 \div 2.4 = 33.3$ microgram/hour	Buprenorphine patch 35 microgram per hour

References

Berkshire Adult Palliative Care Guidelines Best Practice Document V3 2019
 Sue Ryder Anticipatory 'Just in case' Prescribing Guidelines Dec 2021.
 OUH Palliative Care Opioid Conversion Chart V1.1 Jan 2022

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